

Policy on Safe Dispensing of Medications

«Hospital Name»

Document control

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Prepared by	Pharmacy In-Charge Name: _____ Signature: _____		
Reviewed by	Quality Coordinator Name: _____ Signature: _____		
Approved by	Medical Superintendent Name: _____ Signature: _____		

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Statement of intent

Medications are dispensed only against a valid prescription, checked before dispensing, correctly labelled, verified before a high-risk order is dispensed, and recalled or expired medications never reach a patient.

1. Purpose

This policy explains how «Hospital Name» dispenses medications safely, handles recalls and near-expiry stock, labels dispensed medications, verifies high-risk orders before dispensing, and manages medication returns.

This policy does not cover storage or prescription of medications in detail — those are covered in other hospital policies.

2. Scope

This policy applies to all pharmacy staff involved in dispensing medications at «Hospital Name».

3. Policy standards

«Hospital Name» dispenses medications only against a valid prescription or order, checked for generic composition, formulation, expiry, and strength, across both bulk and retail pharmacy. Medication recalls and near-expiry stock are handled through defined systems,

every dispensed medication is labelled, high-risk orders are verified before dispensing, and medication returns follow written guidance.

Staff follow the written guidance below and keep the records it requires.

4. Non-negotiable rules

1. Do not dispense a medication without a valid prescription or medication order, except for over-the-counter drugs, and do not dispense without checking generic composition, formulation, expiry date, and strength where applicable. Do not sell physicians' samples.
2. Do not leave a recalled medication in usable stock, and do not fail to report any internally identified recall to the appropriate regulatory authority.
3. Do not let a beyond-expiry-date medication remain available in clinical stock.
4. Do not dispense a medication that is unlabelled, recalled, expired, or a high-risk order that has not been verified for dose, frequency, and route.
5. Do not accept a returned medication without written guidance on which medications are accepted and the minimum conditions they must meet.

5. What we do

5.1 Dispense medications safely

Written guidance governs the safe dispensing of medications. Medications are dispensed only against a valid prescription or medication order, except for over-the-counter drugs. Before dispensing, medications are checked for generic composition, formulation, expiry date, and — where applicable — strength. This applies to both bulk and retail pharmacy. Physicians' samples are never sold.

5.2 Handle medication recalls

«Hospital Name» has an established system for medication recalls, whether triggered by a regulatory authority, the manufacturer, or internal feedback — for example, noticing a visible contaminant in an IV fluid bottle. Where a recall arises from internal feedback, the appropriate regulatory authority is also informed. A record is kept whenever a recall occurs.

5.3 Handle near-expiry medications

«Hospital Name» defines what counts as "near expiry" — for example, three months before the expiry date — and has a mechanism to withdraw near-expiry stock before it goes past that date. No beyond-expiry-date medication is available in usable clinical stock.

5.4 Label dispensed medications

At minimum, every label includes dosage instructions the patient can understand. Where medicines are dispensed as cut strips or from bulk containers, the label also includes the drug name, strength, and expiry date. This applies to both inpatients and outpatients, and to

reconstituted drugs, such as chemotherapy medications. «Hospital Name» could use technology like QR codes on individual medicines to strengthen this process.

5.5 Verify high-risk orders before dispensing

High-risk medications are dispensed only against a written order, verified by staff before dispensing, and in line with applicable statutory requirements.

5.6 Manage medication returns

Written guidance directs how medications are returned to the pharmacy, at minimum covering which medications are accepted for return — defined by inclusion or exclusion — and the minimum conditions for accepting a return, such as matching drug name, strength, batch number, and expiry date to the bill, and no visible damage. It's preferable not to accept a return of any medication with a specific temperature storage requirement once it has left the hospital's premises.

6. Stop-work authority

Do not dispense a medication that is unlabelled, recalled, expired, or a high-risk order that has not been verified for dose, frequency and route.

Stop-work applies to the dispense. Immediate life-saving issue from floor stock in an emergency follows the documented after-hours or emergency-dispense rules and is recorded.

The person who stops tells the Pharmacy In-Charge the same shift. Refusing an unsafe dispense is not a disciplinary matter.

7. Governance and responsibility

Role	Responsibility
Medical Superintendent	Accountable for safe dispensing implementation across the organisation.
Pharmacy In-Charge	Owens day-to-day dispensing, recall handling, labelling compliance, high-risk order verification, and medication returns. Receives stop-work escalations the same shift.
Medication Safety Officer	Coordinates notification of internally identified recalls to the appropriate regulatory authority; brings dispensing incidents to the DTC.
Quality Coordinator	Audits adherence to this policy; holds training records.

8. Quality monitoring

Monitoring area	What is monitored
Safe dispensing	Valid prescription or order checked before every dispense; generic composition, formulation, expiry, and strength checked; applies to bulk and retail pharmacy; no physicians' samples sold.
Medication recalls	Established recall system in place; internally identified recalls reported to the appropriate regulatory authority the same shift; records kept on occurrence.
Near-expiry and beyond-expiry stock	Near-expiry threshold defined; withdrawal mechanism in place before the defined date; no beyond-expiry-date medication available in clinical stock.
Labelling	All dispensed medications labelled with dosage instructions; cut strips and bulk-container labels include drug name, strength, and expiry date; applies to inpatients and outpatients; reconstituted drugs (e.g. chemotherapy) labelled.
High-risk order verification	Written order present for every high-risk dispense; staff verification completed before dispensing; statutory requirements met.
Medication returns	Written returns guidance in place; inclusion/exclusion list defined; minimum conditions documented; return records kept.
Stop-work events	Stop-work events logged with trigger, action taken, and outcome.

9. Training and staff acknowledgement

All pharmacy staff involved in dispensing medications shall be familiar with the safe-dispensing guidance, recall and near-expiry procedures, labelling requirements, high-risk order verification, medication-return guidance, and stop-work authority in this policy.

I have read the Policy on Safe Dispensing of Medications of «Hospital Name». I will follow the processes described.

Staff name	Designation	Signature	Date
_____	_____	_____	_____
_____	_____	_____	_____
_____	_____	_____	_____

10. Distribution

This policy shall be available to all pharmacy staff, the Pharmacy In-Charge, the Medical Superintendent, the Medication Safety Officer, and the Quality Coordinator.

11. Abbreviations

Abbreviation	Meaning
DTC	Drug and Therapeutics Committee
IV	Intravenous
MOM	Management of Medication (NABH Hospitals chapter)
MSO	Medication Safety Officer
NABH	National Accreditation Board for Hospitals and Healthcare Providers
OTC	Over-the-counter
QR	Quick Response (code)

12. Traceability table

This table is an index. It is not how the policy is organised. An asterisk in the Level column means documentation of the process is required.

Objective Element	Level	Traceability to this policy
MOM.6.a	Commitment*	Sections 3 and 5.1 address the written safe-dispensing guidance, valid prescription or order requirement, pre-dispense checks (generic composition, formulation, expiry, strength), applicability to bulk and retail pharmacy, and the prohibition on selling physicians' samples.
MOM.6.b	Commitment*	Section 5.2 addresses the established recall system, the requirement to inform the appropriate regulatory authority when a recall is internally identified, and the record-keeping requirement on every recall occurrence.
MOM.6.c	Commitment*	Section 5.3 addresses the definition of 'near expiry,' the withdrawal mechanism for near-expiry stock, and the absolute requirement

		that no beyond-expiry-date medication is available in usable clinical stock — near-expiry and beyond-expiry are treated as distinct stages.
MOM.6.d	CORE*	Sections 3 and 5.4 address universal labelling scope — both inpatients and outpatients, and reconstituted drugs such as chemotherapy medications. Cut strips and bulk containers carry drug name, strength, and expiry date in addition to dosage instructions. This resolves the Guidebook internal inconsistency in favour of universal scope (not outpatient-only). This OE is also a stop-work trigger in Section 6.
MOM.6.e	CORE	Sections 3 and 5.5 address the written-order requirement for high-risk medications, staff verification before dispensing, and statutory requirements. This is also a stop-work trigger in Section 6.
MOM.6.f	Commitment*	Section 5.6 addresses the written medication-return guidance, the inclusion/exclusion definition of accepted medications, and the minimum conditions for accepting a return.

13. Required Records/Evidence Checklist

Safe dispensing — MOM.6.a (Commitment*)

1. Written safe-dispensing guidance in place.
2. Dispense records showing valid prescription or order checked before every dispense.
3. Pre-dispense check records covering generic composition, formulation, expiry date, and strength.

4. Evidence the process applies to both bulk and retail pharmacy.
5. Confirmation physicians' samples are not sold.

Medication recalls — MOM.6.b (Commitment*)

1. Established recall system documentation.
2. Recall file with trigger, batches affected, quarantine action, and recovery steps.
3. Regulatory-authority notification records for any internally identified recalls.
4. Records kept on every recall occurrence.

Near-expiry and beyond-expiry — MOM.6.c (Commitment*)

1. Organisation's defined 'near-expiry' threshold (e.g. three months).
2. Withdrawal records for near-expiry stock.
3. Evidence that no beyond-expiry-date medication is available in usable clinical stock.

Labelling — MOM.6.d (CORE*)

1. Sample labelled dispensed medications showing dosage instructions the patient can understand.
2. Cut-strip and bulk-container labels showing drug name, strength, expiry date, and dosage instructions.
3. Evidence that labelling applies to both inpatients and outpatients.
4. Evidence that labelling applies to reconstituted drugs (e.g. chemotherapy medications).

High-risk order verification — MOM.6.e (CORE)

1. Written-order records for every high-risk medication dispense.
2. Staff-verification records before dispensing (second-person check or equivalent).
3. Statutory compliance records where applicable.

Medication returns — MOM.6.f (Commitment*)

1. Written medication-return guidance.
2. Inclusion/exclusion list of medications accepted for return.
3. Minimum-conditions record (drug name, strength, batch, expiry matching bill; no visible damage).
4. Return records.

14. References

1. National Accreditation Board for Hospitals and Healthcare Providers. NABH Accreditation Standards for Hospitals, 6th Edition. MOM.6.

Disclaimer

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